

TABLE 1

INSTRUMENTAL VARIABLE ESTIMATES OF INTENSIVE MANAGEMENT AND SPENDING ON
ONE-YEAR SURVIVAL BY CLINICAL APPROPRIATENESS OF PATIENT

SAMPLE	INSTRUMENTAL VARIABLE ESTIMATES OF		
	Impact of Cath		Impact of \$1,000 on One-Year Survival
	On One-Year Survival (1)	On One-Year Cost (\$1,000s) (2)	
A. All patients ($N = 129,895$)	.142 (.036)	9.086 (1.810)	.016 (.005)
B. By cath propensity:			
Above the median ($N = 64,799$)	.184 (.034)	4.793 (1.997)	.038 (.017)
Below the median ($N = 65,096$)	.035 (.083)	17.183 (3.204)	.002 (.005)
Difference	.149 (.090)	-12.39 (3.775)	.036 (.018)
C. By age:			
65-80 ($N = 89,947$)	.171 (.037)	6.993 (1.993)	.024 (.009)
Over 80 ($N = 39,948$)	.016 (.108)	16.026 (2.967)	.001 (.007)
Difference	.155 (.114)	-9.033 (3.574)	.023 (.011)

NOTE.—Cath propensity is an empirical measure of patient appropriateness for intensive treatments. We define this measure by using fitted values from a logit model of the receipt of cardiac catheterization on all the CCP risk adjusters. Differential distance (measured as the distance between the patient's zip code of residence and the nearest catheterization hospital minus the distance to the nearest hospital) is the instrument. Each model includes all the CCP risk adjusters, and the standard errors are clustered at the level of each HRR.